

AL Medication Validation

Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Assisted living residents who are aware of their medications may self-administer their medications. Nothing in this policy precludes the community from utilizing licensed nurses to administer medications to residents who are capable of self-administering.

Definitions

Aware

- The resident can maintain possession and control of their medications, and self-administer medications, without creating an unreasonable risk to health and safety; **or**
- The resident has a reasonable lay person's understanding of the unit dose packaging system in use by the community such that the resident could likely protect themselves from medication errors if unit dose packages are brought to the resident by community staff.

Guidelines

All residents receiving medication assistance or administration will be validated on a monthly basis to determine their awareness of their medications.

- Pharmacy will print "Medication Validation" on last line of last page of each resident's Medication Administration Record (MAR).

- The schedule for Medication Validation is as follows:

1 st Monday	1 st Tuesday	1 st Wednesday	1 st Thursday	1 st Friday

- The nurse on duty on first or second shift will validate the resident's awareness per the definition above.
 - The Resident Care Director will make assignments for validating.
- If the resident is aware, the MAR should be initialed in the box for that shift.
 - The resident will continue to be evaluated monthly.
- If the resident is not aware, the initials should be circled, and an explanation written on the back of the MAR.
 - Medication teaching will be initiated. A progress note should be written. The

resident will be placed on a weekly validation schedule until they are aware of their medications or are determined to require administration.

-Once the resident demonstrates awareness of their medications, they will be returned to a monthly schedule for validation. A progress note should be written.



AL-Monthly Care Review

Year _____

Resident _____ Apt # _____

Month	Weight	BP	Falls	Incidents	Behavior Issues	Significant Health Changes	Med Changes or Adverse Reactions	Other
January								
February								
March								
April								
May								
June								
July								
August								
September								
October								
November								
December								

Monthly Narrative Summaries

DATE: _____ Evaluation Review and Plan of Care Outcome Notes

Signature _____

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AL Reporting Requirements

Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Each licensee (hereafter referred to as the Community) will furnish to the ADPH licensing agency any reports the Department requires. These reporting requirements apply to both ALFs and SCALFs. the community or a secured area without supervision or the knowledge of staff.

Guidelines

Twenty Four Hour /Fax Reporting Requirements

The following events require a fax line report within 24 hours.

- Fracture or injury resulting in: hospitalization, death, EMS activation, visit to the emergency room
- Elopement by a resident
- Sexual contact or a report of sexual conduct between a resident and a staff member of the community. Sexual conduct or report of sexual contact between a resident and a visitor or another resident when the sexual contact is not consensual, or the resident is incapable of consenting to sexual contact.
- Physical abuse, verbal abuse, emotional abuse or a report of such abuse, directed at a resident by a staff member of the community or visitor, and injury such as extensive bruising, pain, or injury that is not consistent with actions necessary in providing day to day care to a resident
- Resident on resident physical abuse resulting in injury
- A fire, earthquake, storm act of God, or other occurrence that causes physical damage to the building in which the community is located or that results in the evacuation or partial evacuation of the community
- Intentional self-inflicted injury, suicide, or suicide attempt by a resident
- Any unplanned event that results in media attention
- A medication error, including overdose that requires a resident being hospitalized
- Ingestion by a resident of a toxic substance requiring medical intervention
- A malfunction of the sprinkler system, fire alarm system or door locking device

BEHAVIOR SCREEN (BS)**Date:** _____ **Patient's Name:** _____**Informant:** _____Explain and describe
all "Yes" answers.

In the past six months has the patient been:

Disruptive/Loud:

☐ No☐ Yes

Socially Objectionable: (ex. Spitting/stripping/masturbating, etc.)

☐ No☐ Yes

Demanding:

☐ No☐ Yes

Verbally Aggressive:

☐ No☐ Yes

Hitting/Fighting/Scratching:

☐ No☐ Yes

Destructive to Self or Property:

☐ No☐ Yes

Wandering/Pacing:

☐ No☐ Yes

Delusional: (Express false ideas)

☐ No☐ Yes

Hallucinations: (See/hear things not there)

☐ No☐ Yes

On Drugs for Behavior: (Drug/Reason/How Long)

☐ No☐ Yes

Has Patient Lost Weight in the Past Few Months: (if yes, how much?)

☐ No☐ Yes

Geriatric Depression Scale (Long Form)

Patient's Name: _____

Date: _____

Instructions: Choose the best answer for how you felt over the past week.

No.	Question	Answer	Score
1.	Are you basically satisfied with your life?	YES / NO	
2.	Have you dropped many of your activities and interests?	YES / NO	
3.	Do you feel that your life is empty?	YES / NO	
4.	Do you often get bored?	YES / NO	
5.	Are you hopeful about the future?	YES / NO	
6.	Are you bothered by thoughts you can't get out of your head?	YES / NO	
7.	Are you in good spirits most of the time?	YES / NO	
8.	Are you afraid that something bad is going to happen to you?	YES / NO	
9.	Do you feel happy most of the time?	YES / NO	
10.	Do you often feel helpless?	YES / NO	
11.	Do you often get restless and fidgety?	YES / NO	
12.	Do you prefer to stay at home, rather than going out and doing new things?	YES / NO	
13.	Do you frequently worry about the future?	YES / NO	
14.	Do you feel you have more problems with memory than most?	YES / NO	
15.	Do you think it is wonderful to be alive now?	YES / NO	
16.	Do you often feel downhearted and blue?	YES / NO	
17.	Do you feel pretty worthless the way you are now?	YES / NO	
18.	Do you worry a lot about the past?	YES / NO	
19.	Do you find life very exciting?	YES / NO	
20.	Is it hard for you to get started on new projects?	YES / NO	
21.	Do you feel full of energy?	YES / NO	
22.	Do you feel that your situation is hopeless?	YES / NO	
23.	Do you think that most people are better off than you are?	YES / NO	
24.	Do you frequently get upset over little things?	YES / NO	
25.	Do you frequently feel like crying?	YES / NO	
26.	Do you have trouble concentrating?	YES / NO	
27.	Do you enjoy getting up in the morning?	YES / NO	
28.	Do you prefer to avoid social gatherings?	YES / NO	
29.	Is it easy for you to make decisions?	YES / NO	
30.	Is your mind as clear as it used to be?	YES / NO	
TOTAL			

This is the original scoring for the scale: One point for each of these answers.

Cutoff: normal-0-9; mild depressives-10-19; severe depressives-20-30.

1. NO	6. YES	11. YES	16. YES	21. NO	26. YES
2. YES	7. NO	12. YES	17. YES	22. YES	27. NO
3. YES	8. YES	13. YES	18. YES	23. YES	28. YES
4. YES	9. NO	14. YES	19. NO	24. YES	29. NO
5. NO	10. YES	15. NO	20. YES	25. YES	30. NO

Yesavage JA, Brink TL, Rose TL, et al. Development and validation of a geriatric depression screening scale: a preliminary report. *J Psychiatr Res* 1983; 17:37-49.

MS APHASIA SCREENING TEST

Name: _____
Date: _____ DOB: _____
Handedness: _____
Education: _____
Date of Onset: ____/____/____ Side 1

Naming (2 points each; present object and ask "What is this called?")

- 1) ___ Pen
- 2) ___ Hand (point to both sides of your hand)
- 3) ___ Thumb
- 4) ___ Watch
- 5) ___ Ceiling (also accept light)

Automatic Speech (2 = correct; 0 incorrect; for items 3-5 say "Finish these sentences for me.")

- 1) ___ Count to ten (1 = cueing required)
- 2) ___ Tell me the days of the week (1 = cueing required)
- 3) ___ Three strikes and you're _____.
4) ___ I pledge allegiance to the _____.
5) ___ The phone is off the _____.

Repetition (2 points correct; 0 = incorrect; Say "Repeat these words")

- 1) ___ pot
- 2) ___ carrot
- 3) ___ alphabet
- 4) ___ under the old wooden bridge
- 5) ___ The silver moon hung in the dark sky

Yes/No Responses (2 pts; "y"=yes; "n" = no; "I'm going to ask some questions; just tell me yes or no")

- 1) ___ Is your name **Johnson** (change if last name is Johnson)
- 2) ___ Is your name _____? (insert correct last name)
- 3) ___ Do you live in **Rhode Island**?
- 4) ___ Do you live in _____? (insert correct state)
- 5) ___ Do you wear a glove on your foot?
- 6) ___ Am I touching my eye (clinician touches his/her nose)?
- 7) ___ Does Monday come before Tuesday?
- 8) ___ Does summer come after spring?
- 9) ___ Is a chicken bigger than a spider?
- 10) ___ Do you put your shoe on before your sock?

Object Recognition in a Field of Five (2 pts each; last 3 objects can use following possible items:

Book, paper, pen, photo, coin, name badge, cup)

___ Watch ___()
___ Keys ___()
___()

"I want to show you some things, point to them as I call them out."

Following Instructions (2 points each)

- 1) ___ Point to your nose
- 2) ___ Open your mouth
- 3) ___ With your left hand, point to your right eye.
- 4) ___ Point to the floor, then point to your nose.
- 5) ___ Before opening your mouth, touch your ear.

MS APHASIA SCREENING TEST

Name: _____
Observations: _____

Side 2

Reading Instructions (2 points each)

- 1) Open your mouth ("Read this aloud and do what it says")
- 2) Make a fist (Now read the next few silently to yourself and do what it says")
- 3) Point to the floor, then point to the ceiling.
- 4) With your right hand, point to your left knee. (Alternative: With your left hand, point to your right knee)
- 5) Point to your left ear after you make a fist.

Verbal Fluency Dictation: [SIDEWALK] write all words that the patient verbalizes and code unintelligible utterances with a dash ("-") ; OR TAPE PT RESPONSE AND TRANSCRIBE AFTERWARDS

INSTRUCTION: Show the photo (10 seconds) and say "Look at this picture for a while (pause) Now tell me everything that you can about this picture. keep talking until I tell you to stop". immediately start timing for 10 seconds..

____ Number of Intelligible Verbalizations

Subscale score Conversion: 0-5 intelligible verbalizations = 0; 5 - 10 = 5; 11+ = 10 points.

Writing/Spelling (2 points each) "Now I would like for you to write some words for me, spell ____"

- 1 sit
- 2 twist
- 3 airplane
- 4 computer
- 5 under the black bridge

Expressive Index		Receptive Index		Total Index	
Naming	____/10	Yes/No Accuracy	____/20	Expressive	____/50
Automatic Speech	____/10	Object Recognition	____/10	Receptive	____/50
Repetition	____/10	Following Instructions	____/10		
Writing	____/10	Reading Instructions	____/10		
Verbal Fluency	____/10				

Expressive Subscale ____/50 Receptive Subscale ____/50 Total Score ____/100

Optional Ratings (indicate presence "+" or absence "-")

Dysarthria: _____
Paraphasia: _____
Perseveration: _____
Oriented: _____

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AL SCALF RN Assessment

Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

To meet SCALF Rules and Regulations of Registered Nurse Assessment of residents' nursing issues identified through the assessment process to include falls, weight loss, behavior, medication changes and reactions, and elopement history.

Guidelines

A Registered Nurse (RN) will complete a nursing assessment (see attached RN Assessment/Nursing Plan of Care form) form on admission, monthly, post hospitalization/rehabilitation, and at any change in resident's condition. This form will complement the Resident Health Evaluation to identify needs of the individual resident.

- This assessment will complement the other Merrill Gardens tools as identified in the *Assisted Living Evaluation and Service Planning* policy
- Other assessments will be used as defined in Alabama SCALF Rules and Regulations to include but not limited to: Mini Mental, Behavior Screening, PSMS, Geriatric Depression Scale, Aphasia Screen, & Psychotropic Drug Usage.
- The assessment outcomes will then be used to identify any nursing care plan needs. These needs will be documented on the RN Assessment form and operationalized on the Service Plan. This process communicates to the staff processes they monitor and document against.
- Outcomes are then assessed and documented on the nursing progress notes and monthly as needed on the *RN Assessment* form.



AL- SCALF RN Resident Care Plan

Resident Name:

Apt:

NEED	ACTION	RESPONSIBLE PARTY
BATHING		☐ Caregiver
		☐ Nurse
		☐
AMBULATION/ TRANSFERS		☐ Caregiver
		☐ Nurse
		☐
DRESSING		☐ Caregiver
		☐ Nurse
		☐
GROOMING		☐ Caregiver
		☐ Nurse
		☐
TOILETING		☐ Caregiver
		☐ Nurse
		☐
EATING/DIET		☐ Caregiver
		☐ Nurse
		☐
PERSONAL SAFETY		☐ Caregiver
		☐ Nurse
		☐
SOCIALIZATION		☐ Garden House Supervisor
		☐ Caregiver
		☐ Nurse
MEDICATIONS		☐
		☐ Nurse
		☐
		☐
		☐



AL-SCALF RN Comprehensive Assessment Information

Resident: _____ DOB: _____ Date of Assessment: _____

Check reason for this assessment: ☐ Move-in ☐ Post Hospitalization: Reason for Hospitalization: _____

Change in Condition:(circle one) Falls, Weight loss, Elopement, Behavioral Symptoms,
Medication reaction, Other: _____

Diagnosis: _____

List of Medications:

Neurological System

Oriented to: Person _____, Place _____, Time _____ Alert _____
Lethargic _____ Confused _____ Verbal Response _____ Movement _____ Paralysis/paresis _____

Cardiovascular System

BP _____ P(A) _____ P(P) _____ SOB: _____ at rest _____ on exertion _____ Heart Sounds- _____ WNL _____ ABL
Edema: _____ Chest pain: _____ Relief for Chest pain: _____

Pulmonary System

Respirations: _____ WNL _____ Labored _____ Cough _____ Sputum: Amt _____ Color _____
Breathing Tx: _____ Oxygen: _____

astrointestinal System

Appetite: _____ Diet _____ Bowel Sounds present: _____ Abd: Soft _____ Distended _____
Bowel Habits: _____ Continent: Y _____ N _____ Last BM _____ Dentures _____

Genitourinary System

Continent: Y _____ N _____ Briefs: _____ Pads _____ Urine color _____
Urinary c/o (freq, burning) _____

Musculoskeletal System

Strength: U/E _____ L/E _____ ROM: U/E _____ L/E _____
Posture: _____ Gait: _____ Ambulation _____
_____ Walker _____ Cane _____ W/C

Integumentary System (reported and visible skin areas)

Breakdown Y _____ N _____ Dec/Ulcer: _____ Bruising: _____ Rash _____
Skin tear(s) _____ Other: _____ Any treatments: _____

Behavior/Mental Status

Changes: _____ Behavioral _____ Mental Status _____ Type: _____
Recent changes: _____

Responsible Party Notification(who): _____ Date: _____

Physician Notification- Name: _____ Date: _____

Physician Recommendations: _____

RN Signature: _____

AL- SCALF RN Monthly Assessment/Review

Resident _____

Apt. _____

Use the following area to document the monthly review of the comprehensive assessment until another comprehensive assessment is done; then use a new form. A new comprehensive assessment is to be done at least every six months along with the Service Plan biannual review.

Month	Weight	BP	Falls	Incidents	Elopements	Behavior. Problems	Significant Health Changes Y/N	Medication changes or adverse reactions-	Other

DATE: _____ Assessment Review and Plan of Care Outcome Notes

Comp Assess Needed? Y ___ N ___

RN Signature _____

DATE: _____ Assessment Review and Plan of Care Outcome Notes

Comp Assess Needed? Y ___ N ___

RN Signature _____

DATE: _____ Assessment Review and Plan of Care Outcome Notes

Comp Assess Needed? Y ___ N ___

RN Signature _____

DATE: _____ Assessment Review and Plan of Care Outcome Notes

Comp Assess Needed? Y ___ N ___

RN Signature _____

DATE: _____ Assessment Review and Plan of Care Outcome Notes

Comp Assess Needed? Y ___ N ___

RN Signature _____

